Pharmaniaga Loratadine Tablet 10mg

DESCRIPTION
White, oval-shaped biconvex tablet with Pharmaniaga icon on one side and scored on the other.

COMPOSITION
Each tablet contains Loratadine (micronised) 10 mg.

PHARMACODYNAMICS
Loratadine, the active ingredient is a tricyclic antihistamine with selective, peripheral H_1-receptor activity. It is a piperidine derivative related to azatadine and has little central sedative or antimuscarinic activity. Loratadine has no clinically significant sedative or anticholinergic properties in the majority of the population and when used at the recommended dosage. During long-term treatment there were no clinically significant changes in vital signs, laboratory test values, physical examinations or electrocardiograms. Loratadine has no significant H_2-receptor activity. It does not inhibit norepinephrine uptake and has practically no influence on cardiovascular function or on intrinsic cardiac pacemaker activity.

PHARMACOKINETICS
Absorption: Loratadine is rapidly and well-absorbed. Concomitant ingestion of food can delay slightly the absorption of loratadine but without influencing the clinical effect. The bioavailability parameters of loratadine and of the active metabolite are dose proportional.

Distribution: Loratadine is highly bound (97% to 99%) and its active major metabolite desloratadine (DL) moderately bound (73% to 76%) to plasma proteins. In healthy subjects, plasma distribution half-lives of loratadine and its active metabolite are approximately 1 and 2 hours respectively.

Biotransformation: After oral administration, loratadine is rapidly and well absorbed and undergoes an extensive first pass metabolism, mainly by CYP3A4 and CYP2D6. The major metabolite-desloratadine (DL)- is pharmacologically active and responsible for a large part of the clinical effect. Loratadine and DL achieve maximum plasma concentrations (T_{max}) between 1-1.5 hours and 1.5-3.7 hours after administration, respectively.

Elimination: Approximately 40% of the dose is excreted in the urine and 42% in the faeces over a 10 day period and mainly in the form of conjugated metabolites. Approximately 27% of the dose is eliminated in the urine during the first 24 hours.

Less than 1% of the active substance is excreted unchanged in the active form, as loratadine or DL.

The mean elimination half-lives in healthy adult subjects were 8.4 hours (range = 3 to 20 hours) for loratadine and 28 hours (range = 8.8 to 92 hours) for the major active metabolite.

Renal impairment

In patients with chronic renal impairment, both the AUC and peak plasma levels (C_{max}) increased for loratadine and its active metabolite as compared to the AUCs and peak plasma levels (C_{max}) of patients with normal renal function. The mean elimination half-lives of loratadine and its active metabolite were not significantly different from that observed in normal subjects. Haemodialysis does not have an effect on the pharmacokinetics of loratadine or its active metabolite in subjects with chronic renal impairment.

Hepatic impairment

In patients with chronic alcoholic liver disease, the AUC and peak plasma levels (C_{max}) of loratadine were double while the pharmacokinetic profile of the active metabolite was not significantly changed from that in patients with normal liver function. The elimination half-lives for loratadine and its metabolite were 24 hours and 37 hours, respectively, and increased with increasing severity of liver disease.

Elderly

The pharmacokinetic profile of loratadine and its active metabolite is comparable in healthy adult volunteers and in healthy geriatric volunteers.

INDICATION
Loratadine is indicated for the relief of symptoms associated with allergic rhinitis e.g. sneezing, nasal discharge (rhinorrhea) and itching, as well as ocular itching and burning. Also indicated for relief of symptoms and signs of chronic urticaria and other allergic dermatological disorders.

CONTRAINDICATIONS
Loratadine is contraindicated in patients who are hypersensitive to loratadine.

ADVERSE REACTIONS
In paediatric population, children aged 2 through 12 years, common adverse reactions reported were headache, nervousness and fatigue. The most frequent adverse reactions in adults and adolescents were somnolence, headache, increased appetite and insomnia.

Autonomic Nervous System: Altered lacrimation, altered salivation, flushing, hypoesthesia, impotence, increased sweating, thirst.

Body As A Whole: Angioneurotic oedema, asthenia, back pain, blurred vision, chest pain, earache, eye pain, fever, leg cramps, malaise, rigors, tinnitus, viral infection, weight gain.

Cardiovascular System: Hypertension, hypotension, palpitations, supraventricular tachyarrhythmias, syncope, tachycardia.

Central and Peripheral Nervous System: Blepharospasm, dizziness, dysphonia, hypertonia, migraine, paresthesia, tremor, vertigo, headache, nervousness, fatigue, somnolence, convulsion.

Gastrointestinal System: Altered taste, anorexia, constipation, diarrhea, dyspepsia, flatulence, gastritis, hiccup, increased appetite, nausea, stomatitis, toothache, vomiting, dry mouth, loose stools.

Musculoskeletal System: Arthralgia, myalgia.

Psychiatric: Agitation, amnesia, anxiety, confusion, decreased libido, depression, impaired concentration, insomnia, irritability, paroniria.

Reproductive System: Breast pain, breast enlargement, dysmenorrhea, menorrhagia, vaginitis.

Respiratory System: Bronchitis, bronchospasm, coughing, dyspnea, epistaxis, haemoptysis, laryngitis, nasal dryness, pharyngitis, sinusitis, sneezing.

Skin and Appendages: Dermatitis, dry hair, dry skin, photosensitivity reaction, pruritus, purpura, urticaria, erythema multiforme, rash, alopecia.

Immune System: Hypersensitivity reactions (including angioedema and anaphylaxis.

Urinary System: Altered micturition, urinary discoloration, urinary incontinence, urinary retention.

Hepatobiliary System: Abnormal hepatic function, including jaundice, hepatitis, and hepatic necrosis.

Blood and Lymphatic System: Thrombocytopenia.

WARNINGS AND PRECAUTIONS
Loratadine should be administered with caution in patients with severe liver impairment.

This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

The administration of loratadine should be discontinued at least 48 hours before skin tests since antihistamines may prevent or reduce otherwise positive reactions to dermal reactivity index.

Paediatric use:
The safety and effectiveness of loratadine in paediatric patients under 2 years of age have not been established.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES
No impairment occurred in patients receiving loratadine. However, patients should be informed that very rarely some people experience drowsiness, which may affect their ability to drive or use machines.

PREGNANCY AND LACTATION
Pregnancy
Safe use of loratadine during pregnancy has not been established; therefore, use only if potential benefit justifies potential risk to the foetus.

Lactation
Since loratadine is excreted in breast milk and because of the increased risk of antihistamines for infants, particularly newborns and premature infants, a decision should be made whether to discontinue nursing or discontinue the drug.

DRUG INTERACTIONS
When administered concomitantly with alcohol, loratadine has no potentiating effects as measured by psychomotor performance studies.

Potential interaction may occur with all known inhibitors of CYP3A4 or CYP2D6 resulting in elevated levels of loratadine which may cause an increase in adverse events. Increase in plasma concentrations of loratadine has been reported after concomitant use with ketoconazole, erythromycin and cimetidine but without clinically significant changes (including electrocardiographic).

DOSAGE AND ADMINISTRATION
Adults and children over 12 years of age:
One tablet (10 mg) once daily. The tablet may be taken without regard to mealtime.

Children 6 to 12 years of age with:
Body weight more than 30 kg - 10 mg once daily.

Efficacy and safety of loratadine in children under 2 years of age has not been established.

Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5mg once daily or 10mg every other day is recommended.

ROUTE OF ADMINISTRATION
Oral.

OVERDOSAGE
Overdosage with loratadine increase the occurrence of anticholinergic symptoms. Somnolence, tachycardia and headache have been reported with overdoses. In the event of overdosage, general symptomatic and supportive measures should be initiated promptly and maintained for as long as necessary.

Treatment of overdosage would reasonably consist of emesis (ipecac syrup), except in patients with impaired consciousness, followed by the administration of activated charcoal to absorb any remaining drug. If vomiting is unsuccessful, or contraindicated, gastric lavage should be performed with normal saline. Saline cathartics may also be of value for rapid dilution of bowel contents. Loratadine is not eliminated by haemodialysis. It is also not known if loratadine is eliminated by peritoneal dialysis.

Medical monitoring of the patient is to be continued after emergency treatment.

STORAGE CONDITIONS
Store below 30°C.
Keep container tightly closed.
Protect from light and excessive moisture.

SHELF LIFE
Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION
In bottle of 500 tablets (for export only).
In blisters of 10, 100 and 250 tablets.

Date of Revision: 23rd August 2024

PRODUCT REGISTRATION HOLDER/
MANUFACTURER:
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